FREE GUIDE

The Consultation Bloodwork Brief

What to test. How to prepare for the draw. What to gather before any protocol conversation.

Chris Broomhead
calendly.com/chrisbroomheadconsultation

Educational information only. Not medical advice.

Why bloodwork comes first

A protocol written without data is a guess.

Most people start with the compound. The right starting point is your markers. They show where you are, what needs attention first, and how to measure whether anything you change is working.

This brief does three things:

- Lists the markers worth asking for, grouped by system, with what each one measures.
- Explains how to prepare so your results are reliable and comparable over time.
- Gives you the history checklist to complete before a consultation.

It does not tell you what your results mean for you, or what to take. That depends on the full picture. That is what a consultation is for.

WHERE TO GET TESTED

Ask your GP first. Some of these markers are routinely available on the NHS when clinically indicated. Anything not covered can be ordered through a private UK blood testing service, usually as a finger-prick kit or a venous draw at a clinic. Venous draws are more reliable for hormones and lipids.

> **One rule:** retest with the same lab, at the same time of day, under the same conditions. Otherwise you are comparing two different measurements, not two points in time.

The marker panel

What to ask for, and what each marker measures.

METABOLIC HEALTH

MARKER	WHAT IT MEASURES
Fasting glucose	Blood sugar after an overnight fast.
Fasting insulin	How much insulin it takes to hold that glucose level. An early window on insulin sensitivity.
HbA1c	Average blood glucose over roughly the previous two to three months.

CARDIOVASCULAR

Lipid panel	Total cholesterol, LDL, HDL and triglycerides.
ApoB	The number of cholesterol-carrying particles that can enter artery walls.
hs-CRP	High-sensitivity C-reactive protein. A marker of low-grade systemic inflammation.

HORMONES

Total and free testosterone	Total circulating testosterone, and the unbound fraction available to tissue.
SHBG	Sex hormone binding globulin. Binds testosterone and oestradiol, so it changes how much is free.
Oestradiol	The main oestrogen. Relevant for men and women.
LH and FSH	Pituitary signals that drive your own hormone production.
Prolactin	A pituitary hormone that affects libido, mood and other hormones.

Women: record the day of your cycle. Oestradiol and progesterone results depend on timing.

The marker panel, continued

THYROID

MARKER	WHAT IT MEASURES
TSH	Thyroid stimulating hormone. The pituitary's signal to the thyroid.
Free T4 and free T3	The thyroid hormones themselves. T3 is the active form.

LIVER AND KIDNEY

ALT, AST, GGT	Liver enzymes. Raised levels can reflect liver stress. Hard training also raises ALT and AST.
Creatinine and eGFR	Kidney filtration. Creatinine also rises with muscle mass and creatine use.

BLOOD

Full blood count	Haemoglobin, haematocrit, red cells, white cells and platelets.

NUTRIENTS

Ferritin	Iron stores.
Vitamin D (25-OH)	Vitamin D status.
Vitamin B12 and folate	Needed for red blood cell production and nerve function.

OPTIONAL, IF RELEVANT TO YOUR GOAL

IGF-1	Insulin-like growth factor 1. Reflects growth hormone activity.
Morning cortisol	The main stress hormone, measured at its daily peak.

How to prepare for the draw

Small changes in preparation create large changes in results.

- **Morning draw.** Book between 7am and 10am. Testosterone and cortisol are highest in the morning.
- **Fasted.** Nothing but water for 8 to 12 hours beforehand, unless your lab says otherwise.
- **No hard training for 48 hours.** Heavy sessions can raise liver enzymes and creatinine.
- **Hydrate normally.** Dehydration concentrates the blood and skews several markers.
- **Tell the lab about biotin.** High-dose biotin can interfere with some hormone and thyroid tests.
- **Log your timing.** If you are running a protocol, note the time of your last dose relative to the draw.
- **Same conditions every time.** Same lab, same time of day, same preparation.

The history checklist

Have this written down before any consultation.

- ☐ Your primary goal, in one sentence, with a timeframe
- ☐ Everything you are currently running: compound, dose, frequency, start date
- ☐ Everything you have run before, and what happened
- ☐ Prescribed medications and supplements
- ☐ Relevant medical history and any diagnosed conditions
- ☐ Side effects you have noticed, and when they started
- ☐ Training, sleep and diet: a short, honest summary
- ☐ Your bloodwork as a PDF, with the reference ranges shown

What to do with it

Data only helps if someone reads it properly.

Once your bloodwork is back and the checklist is done, you have what a protocol conversation needs.

- **One specific question?** Protocol Design Consultation, 15 minutes, £50.
- **A full review of what you are running?** Private Consultation, 30 minutes, £100. Written protocol summary within 48 hours.
- **A complex case across several systems?** Comprehensive Consultation, 60 minutes, £150. Structured written protocol within 72 hours.
- **Not sure?** Free 15-minute suitability call.

Send your bloodwork and checklist to the email address in your booking confirmation: 48 hours ahead for 30 minutes, 72 hours ahead for 60 minutes.

BOOK

Stop guessing. Get the protocol written down.

calendly.com/chrisbroomheadconsultation

Questions: admin@chrisbroomheadcoaching.com · +44 7521 052843

IMPORTANT

This guide is general educational information. It is not medical advice and does not create a practitioner–patient relationship. It does not diagnose, treat or prevent any condition. Reference ranges vary between laboratories. Always discuss your results and any changes to your health, medication or protocol with a qualified healthcare provider. If you feel unwell, contact your GP or NHS 111. In an emergency, call 999. © 2026 Christopher Broomhead.